

You've already proven the demand is there. Here's how we make it repeatable.

The Schoeman Growth Partnership — social authority, paid acquisition, and a smarter path from attention to patient.

The one thing worth knowing first.

Something in the market has already responded to Dr Gina. Earlier this summer, your clinic's Instagram grew roughly fivefold in a matter of weeks, then faded. That answers the hardest question in marketing: the right people will respond to you.

What's missing is the way to catch that response, qualify it, and hand it — ready — to the follow-up your clinic already has taking shape. This proposal is about building that missing piece: turning your expertise into steady, qualified patient demand.

This didn't come from a template. I studied your digital presence.

Before recommending anything, here's exactly what I reviewed, and what I didn't.

Instagram — analytics

Both accounts, historical performance, follower trends, views, the June breakout, and the role each account plays.

Facebook — analytics

Both pages, and the reality of the organic audiences.

LinkedIn — content

Your posts, topics, openings, formatting and link placement. The content, not the analytics — I don't have LinkedIn's numbers, so nothing here is a LinkedIn performance claim.

How it connects

The parts of your patient journey and communication set-up that matter for where qualified interest should go next.

I'd rather be precise about what I know than impressive about what I don't.

Five things worth your attention.

1 The market has already responded to you. More on this next.

2 Your authority is spread across two accounts (Dr Gina ~2,350 followers; The Schoeman Clinic ~1,515). A chance to give each a clear job, rather than two calendars pulling against each other.

3 LinkedIn has the expertise; the delivery can lift it. Credible, knowledgeable and consistent, with room in the openings, formatting, link placement and reuse. (Content review.)

4 Facebook isn't worth organic effort right now (very small audiences). That's separate from Meta as a paid platform — a different tool for a different job.

5 Right now, attention has nowhere useful to go. The single biggest opportunity, and it's next.

You're not starting from zero.

~300 → ~1,500

followers · peak week ~23,000 views

In June, your clinic's Instagram grew from around 300 followers to roughly 1,500, with a peak week of about 23,000 views. Then visibility fell away.

I'm not going to call that “going viral”, because we don't yet know what caused it, and pretending we do would be guessing with your money.

Here's why it matters: the market has already shown your expertise can attract the right kind of attention. So instead of guessing what to post next, our first job is to determine what drove it, whether it reached the right audience, and how to build a repeatable way of creating that response — then amplify what works.

When someone discovers you today, what happens next?

Someone sees an excellent post from Gina. Then what? Do we learn who they are, understand their problem, follow up, or know if they ever booked?

Today, almost none of that happens. The trail ends at a like, and warm interest quietly disappears.

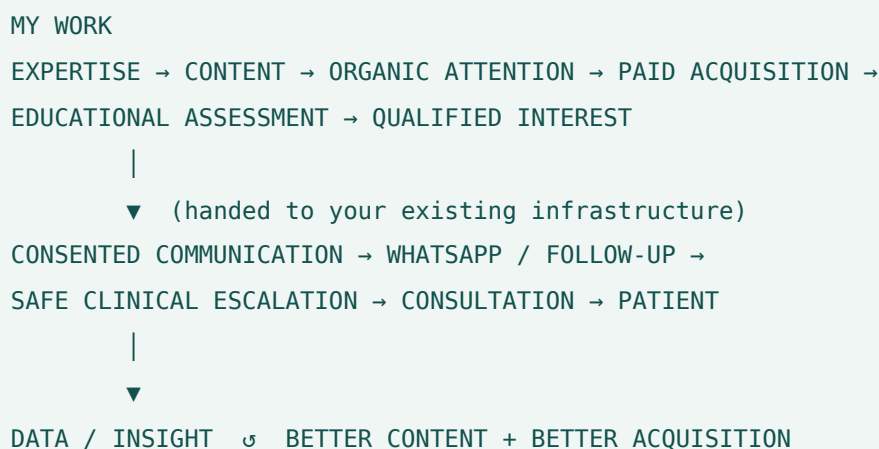
Closing that gap is where the growth is.

The opportunity isn't more content. It's connecting what you already have.

Three layers — and only the first two are what I'm proposing to build. The third already exists in your clinic.

- **Layer 1 — Authority.** Your expertise, turned into content that grows the right audience organically.
- **Layer 2 — Acquisition.** Paid campaigns that amplify what works, an educational assessment that qualifies interest, and a clear next step.
- **Layer 3 — Connection (your existing infrastructure).** The consented communication, WhatsApp follow-up and safe clinical escalation your clinic already has taking shape.

I'm not rebuilding Layer 3. I'm building the growth layer that feeds qualified, ready people into it.



Imagine one of your future patients.

A woman sees a short video from Gina naming something she's quietly lived with for months. Instead of a cold “book a consultation”, she's offered something more useful: a short, educational assessment that helps her understand what might be worth discussing with a clinician.

She learns something. The clinic quietly learns what she's looking for. If she consents, she moves into the follow-up your clinic already runs, and any clinical question goes straight to a clinician.

That's the difference between getting attention and turning it into patients. Advertising stops producing traffic, and starts producing qualified intent.

The assessment educates and guides. It never diagnoses, and it never replaces a clinical consultation.

I've seen this work, not just drawn it on a diagram.

The assessment-to-follow-up approach isn't something I'm proposing because it looks neat on a funnel. I've led marketing inside a health business built on the same principle: paid acquisition feeding a structured assessment and a proper follow-up journey, rather than sending cold traffic straight to a booking.

The lesson stuck with me. In a high-trust field like yours, the strongest results don't come from more clicks. They come from giving interested people the right next step, so they arrive better informed and genuinely ready.

Two accounts, one clear purpose each.

Dr Gina — the authority voice

Your expertise, your perspective, your story. The person patients come to trust.

The Schoeman Clinic — trust & conversion

Your team, your services, the patient journey, and the natural home for introducing your new doctors.

They feed and amplify each other, instead of running as two separate calendars.

First, we confirm what drove June, so the plan is built on evidence.

And this is a full professional service, not just posting: channel strategy and account roles, content pillars and planning, editing and repurposing, captions and creative, publishing and optimisation, and steady testing of hooks, formats and what actually gets discovered.

Advertising, without the gamble.

Paid isn't "boosting posts". Think of it as controlled access to new demand.

Organic shows us what genuinely resonates. Paid then places those proven messages in front of more of the right people. The assessment gives them a useful next step. Your follow-up stops warm interest slipping away. And measurement tells us whether it's actually worth it.

We start small, you control the budget, and we increase spend only when the evidence earns it. Prove before scale.

It isn't “social plus ads”.

- **A social media manager** creates and distributes content.
- **An ads manager** buys and optimises traffic.
- **This partnership** builds your authority organically, learns what resonates, uses paid to amplify proven messages, adds a useful assessment that qualifies interest, connects that interest into your clinic's existing follow-up, and measures what happens — so every cycle improves the next.

That connection, from a first post all the way to a qualified patient conversation, is why this is worth more than either service on its own.

Most marketing starts again every Monday. This gets better every month.

- **Month one:** what earns attention?
- **Month two:** what turns attention into a completed assessment?
- **Month three:** which audiences become real conversations?
- **Month four onward:** which of those lead to consultations?

The lasting asset isn't only the audience we build. It's what the clinic learns about that audience — and no one can copy that.

And because it's built as a repeatable approach, the same foundation can grow with you: new doctors as they join, new services, longevity and men's health, events and partnerships, and in time wider reach. Growth that doesn't have to be reinvented every step.

Your first 90 days.

We don't spend a month deciding what we think — the deep dive already did that. We start acting in week one, test our strongest hypotheses, and improve from real evidence. Throughout, your part stays light: your expertise in focused sessions, clinical approval where needed, the occasional review. I run the rest.

Weeks 1–2 • Turn the audit into action

Apply the LinkedIn improvements we've already identified (openings, formatting, link placement, repurposing); set up the two-account model; begin content from your strongest topics; pull and analyse June's best-performing posts; establish tracking and baseline reporting; confirm clinic capacity and the Assessment V1 concept. *Content improvement begins in week one — we're not waiting a month to post.*

Weeks 3–4 • Build the acquisition layer

Create Assessment V1 (educational, never diagnostic); build the entry journey and tracking; prepare initial paid creative and retargeting audiences; connect the agreed lead handoff into your clinic's existing follow-up (complementary, not rebuilt); keep publishing and optimising.

Weeks 5–8 • Launch and learn

Launch controlled Meta tests on our strongest two to four angles. Recommended minimum media budget: **US\$1,000 / month** — enough to run meaningful tests. It's separate from my fee, paid to the platform, and yours to control and adjust; never scaled automatically. We measure creative response, assessment completion, lead quality, cost per lead, and cost per qualified lead where definable.

Weeks 9–12 • Optimise and decide

Now we can answer the questions that matter: which messages attract the right people, which create real intent, what a qualified opportunity costs, where people drop out, and whether capacity is sufficient. Then we scale what earns it, improve what shows promise, and stop what doesn't. Prove before scale, in practice.

By day 90, you'll have: a working social operating model, an active content engine, a live acquisition journey, real paid-media data, a qualification and assessment process, a clearer view of your acquisition economics, and the evidence for what to scale next. Not a promise of a set number of patients or a guaranteed return — a system producing real answers.

Everything included, and what it's worth.

Most clinics buy these as separate services: a social media manager, an ads manager, a funnel build, and someone to handle tracking. Here they're one partnership.

ONE - TIME

The Growth Build

\$2,500 · approximately £1,850

June breakout investigation · account architecture & roles · content strategy · paid-acquisition strategy · the educational assessment, built · the lead-capture & funnel journey · tracking & measurement · reporting from day one.

MONTHLY · ALL PLATFORMS

The Growth Partnership

\$2,000 / month · approximately £1,480

Social strategy & management (Instagram + LinkedIn) · content, editing, repurposing, captions & creative · publishing, optimisation & testing · Meta campaign management & paid creative · the assessment & funnel, refined · measurement, reporting & monthly strategy.

SEPARATE · YOU CONTROL IT

Advertising budget

recommended minimum \$1,000 / month · about £740

My recommended starting minimum — enough to run meaningful tests and begin learning. Separate from my fee, paid straight to the platforms, and entirely under your control to adjust; never scaled automatically.

Bought separately, your social management alone would usually take a full retainer, and paid campaign management another. Here they work as one. Fees are billed in US dollars; sterling figures are approximate, at today's exchange rate.

Let's map your first 90 days.

You've got the full picture of what I'm recommending. The call isn't to explain it again — it's to switch it on: confirm your capacity, align on priorities and patient value, finalise the scope, and agree a start date.

A focused 30 to 45 minutes, and the proof phase begins.

Let's find a time this week.